Small bowel biopsy

Since dermatitis herpetiformis can be considered the cutaneous counterpart of celiac disease, a proven diagnosis of dermatitis herpetiformis in a patient should serve as a diagnostic indicator for associated small bowel damage. Therefore, small bowel biopsy is generally unnecessary in patients with confirmed dermatitis herpetiformis. Monitoring diet adherence can be achieved through serological testing and clinical observation of skin lesions, as dermatitis herpetiformis lesions typically recur within a few days of gluten ingestion.

Indeed, recent evidence suggests that small bowel biopsy is no longer mandatory for diagnosing celiac disease in certain patient subgroups.

HLA haplotypes testing

Virtually all patients with dermatitis herpetiformis, like those with celiac disease, carry either HLA-DQ2 or HLA-DQ8 haplotypes. The presence of these alleles has nearly 100% sensitivity for dermatitis herpetiformis and a very high negative predictive value—meaning that the absence of these alleles effectively rules out celiac disease.

HLA testing for DQ2 and DQ8 can therefore be a valuable tool in excluding the diagnosis when clinical, serological, or histological findings are inconclusive. However, due to the high prevalence of these alleles in the general population, their specificity for celiac disease or dermatitis herpetiformis is low, limiting their use as a standalone diagnostic test.

Screening for autoimmune diseases and associated conditions

Patients with dermatitis herpetiformis have an increased risk of other immune-mediated disorders, warranting routine screening. Recommended tests include:

- Antithyroid peroxidase antibodies (positive in up to 20% of patients)
- Antigastric parietal cell antibodies (positive in 10–25%)
- Antinuclear antibodies
- Anti-Ro/SSA antibodies

These autoantibodies reflect the underlying autoimmune predisposition associated with celiac disease and dermatitis herpetiformis.

Additionally, screening for: - Thyroid dysfunction (TSH, T3, T4) - Type 1 diabetes (fasting glucose or HbA1c)

is recommended.

Although earlier studies suggested an increased risk of lymphoma, recent data show no significant increase in lymphoma risk in dermatitis herpetiformis patients compared to the general population. Nonetheless, clinicians should remain vigilant for signs of intestinal or extraintestinal lymphoma.

Screening first-degree relatives for celiac disease

First-degree relatives of patients with dermatitis herpetiformis or celiac disease have a higher risk of developing celiac disease. While screening symptomatic relatives is clearly beneficial, there is limited evidence supporting routine screening of asymptomatic first-degree relatives.

Therapy

Gluten-free diet (GFD)

A gluten-free diet (GFD) is the cornerstone of treatment for both celiac disease and dermatitis herpetiformis, as both the intestinal and cutaneous manifestations are gluten-dependent.

- Gastrointestinal symptoms typically improve rapidly on a GFD.
- Skin lesions resolve more slowly, with an average of 2 years required for complete clearance.
- Skin rash invariably recurs within 12 weeks of gluten reintroduction.
- IgA deposits at the dermal-epidermal junction may take several years to disappear with strict dietary adherence and reappear upon gluten exposure, often preceding or accompanying rash recurrence.

The toxic cereals belong to the tribe *Triticaceae* and include: - Wheat - Rye - Barley

Historically, the gluten-free diet excluded wheat, barley, rye, and oats (mnemonic: BROW). However, recent studies indicate that pure, uncontaminated oats (from the *Avena* genus) are safe for most patients with celiac disease or dermatitis herpetiformis.

Important: Only oats that are certified free from contamination with wheat, barley, or rye should be consumed, as most commercial oats are cross-contaminated.